

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 49.9 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 2 Gynecology Ward	Room/Bed: 1 B 2 Gynecology Ward/ 1 B 2 Bed 19	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 28-Mar-2023 14:03	D.O.D. : 03-Apr-2023	D.O. Print Report: 08-Feb-2026 15:26

PROVISIONAL DIAGNOSIS:

46 YEARS/ P1L1A1/ TL NOT DONE/ LCB 22 YEARS AGO/ LARGE INTRAMURAL FUNDAL FIBROID (10.3 X 9.9 X 8.9CM)

FINAL DIAGNOSIS:

46 YEARS/ P1L1A1/ TL NOT DONE/ LCB 22 YEARS AGO / POD 4 OF TOTAL ABDOMINAL HYSTERECTOMY BILATERAL SALPINGOOOPHERECTOMY IVO FIBROID WITH ENDOMETRIOSIS

OPERATION PERFORMED:

TOTAL ABDOMINAL HYSTERECTOMY BILATERAL SALPINGOOOPHERECTOMY IVO FIBROID WITH ENDOMETRIOSIS On 30-Mar-2023

SURGEONS: Dr Pushpawati

ASSISTANT SURGEONS/SENIOR RESIDENTS: Dr Vijaylakshmi, Dr Tithi

TECHNICIANS/NURSES: Nogesh

OPERATIVE FINDINGS: UNDER SPINAL AND EPIDURAL ANESTHESIA PATIENT LAID IN DORSAL LITHOTOMY POSITION AND UNDER ALL ASEPTIC PRECAUTIONS PARTS WERE PAINTED AND DRAPED. LOWER ABDOMINAL 10CM TRANSVERSE INCISION GIVEN . ABDOMEN OPENED IN LAYERS. UTERUS 20 WEEKS SIZE WITH FUNDAL INTRAMURAL FIBROID OF 10 X 10 X 10 CM WITH BILATERAL TUBES AND OVARY ADHERENT TO POSTERIOR UTERINE SURFACE (SUSPICION OF ENDOMETRIOTIC FEATURES) . BILATERAL UTERINE ARTERIES LIGATED. BILATERAL MACKENRODT AND UTEROSACRAL LIGAMENTS IDENTIFIED, CLAMPED AND CUT. BILATERAL CERVICAL CLMAPS APPLIED, CUT AND UTERUS, CERVIX AND BILETRAL TUBES AND OVARIES DELIVERED OUT. VAULT IS CREATED, SUTURED IN CONTINUOUS INTERLOCKING FASHION. ADEQUATE HEMOSTASIS ACHIEVED. MOPS AND INSTRUMENTS COUNT VERIFIED WITH SCRUB NURSE. ABDOMEN CLOSED IN LAYERS WITH VICRYL 1-0 ROUND BODY. SKIN CLOSED IN SUBCUTICULAR FASHION WITH MONOCRYL 2-0 ROUND BODY. STERILE DRY DRESSING DONE. PATEINT WITHSTOOD THE PROCEDURE WELL. INTRAOP BLOOD LOSS 500ML, URINE OUPUT 300ML

CHIEF COMPLAINTS:

DYSMENORRHOEA FOR 1.5 YR

MASS PER ABDOMEN FOR 1.5 YRS

HISTORY OF PRESENT ILLNESS: 46 YEARS/ P1L1A1/ TL NOT DONE/ LCB 22 YEARS AGO CAME WITH THE CHIEF COMPLAINTS OF PAIN DURING MENSES AND MASS PER ABDOMEN SINCE 1.5 YEARS. HER USG DONE OUTSIDE (4/9/21) WAS SUGGESTIVE OF BULKY UTERUS WITH LARGE INTRAMURAL FUNDAL FIBROID (10.3 X 9.9 X 8.9CM). HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT. HER VITALS WERE STABLE AT THE TIME OF ADMISSION.



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



GENERAL EXAMINATION:

GC FAIR

PR- 74/MIN, RR-20/MIN, BP-138/80 MMHG

TEMP-AFEBRILE

PALLOR/ICTERUS/CYANOSIS/CLUBBING/EDEMA ABSENT

SYSTEMIC EXAMINATION:

CVS/CNS/RS-WNL

PA: SOFT, NON TENDER

L/E: 8-20 WEEK,MOBILE SIDE TO SIDE ,VERTICAL MOBILITY RESTRICTED.NO SCAR .

P/S: CERVIX CENTRAL,SMOOTH,DEVIATED TO LEFT,PHYSIOLOGICAL OS PRESENT,NO GROWTH,REGULAR,NORMAL SIZE AND SHAPE,LOOKS HEALTHY,NO ECTROPION/EROSION/BLEEDING/DISCHARGE

VAGINA HEALTHY

P/V: UTERUS 18-20 WEEKS SIZE,GLOBULAR,ENLARGED,REGULAR, MOBILE AND SIDE TO SIDE,VERTICALLY MOBILITY RESTRICTED

B/L FORNICES FREE AND POD FREE AND NON TENDER

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
31-Mar-2023 11:15	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.01	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	1.06	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	1.12	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	14.26	2 - 7 x10 ³ /μL
		Basophil	0.1	00 - 01 %
		Eosinophils	0.1	01 - 06 %
		HCT	37.5	36 - 46 %
		Hemoglobin	11.4	12 - 15 g/dl
		Lymphocytes	6.4	20 - 40 %
		MCH	26.6	27 - 32 pg
		MCHC	30.4	31.5 - 34.5 g/dL
		MCV	87.6	83 - 101 fL
		Monocytes	6.8	2 - 10 %
		Neutrophils	86.6	40 - 80 %
		Platelet Count	301	150 - 410 x10 ³ /μL
		RBC	4.28	3.8 - 4.8 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	18.1	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	16.47	4 - 10 x10 ³ /μL
29-Mar-2023 10:25	ACTIVATED PARTIAL THROMBOPLASTIN	APTT HEADER	22.3	23.2 - 32.4 sec

TIME (APTT)	APTT NOTE	Refer Reports*	-
	clinical details	--	-
COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
	Absolute Basophil Count	0.03	0.02 - 0.10 x10 ³ /μL
	Absolute Eosinophil Count	0.24	0.02 - 0.5 x10 ³ /μL
	Absolute Lymphocyte Count	1.49	1 - 3 x10 ³ /μL
	Absolute Monocyte Count	0.42	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	5.81	2 - 7 x10 ³ /μL
	Basophil	0.4	00 - 01 %
	Eosinophils	3.0	01 - 06 %
	HCT	38.7	36 - 46 %
	Hemoglobin	11.6	12 - 15 g/dl
	Lymphocytes	18.6	20 - 40 %
	MCH	26.9	27 - 32 pg
	MCHC	30.0	31.5 - 34.5 g/dL
	MCV	89.8	83 - 101 fL
	Monocytes	5.3	2 - 10 %
	Neutrophils	72.7	40 - 80 %
	Platelet Count	317	150 - 410 x10 ³ /μL
	RBC	4.31	3.8 - 4.8 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	18.7	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	7.99	4 - 10 x10 ³ /μL
PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.0	-
	PT INR Header	9.9	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: BLOOD GRP- A POSTIVE

USG(4.9.2021)-UTERUS IS BULKY AND LARGE HETEROGENOUSLY HYPOECHOIC SOLID MASS IN BODY AND FUNDAL REGION INDICATING A LARGE INTRAMURAL FIBROID. (10.3 X 9.9 X 8.9 CM)

HOSPITAL COURSE: 46 YEARS/ P1L1A1/ TL NOT DONE/ LCB 22 YEARS AGO CAME WITH THE CHIEF COMPLAINTS OF PAIN DURING MENSES AND MASS PER ABDOMEN SINCE 1.5 YEARS. HER USG DONE OUTSIDE (4/9/21) WAS SUGGESTIVE OF BULKY UTERUS WITH LARGE INTRAMURAL FUNDAL FIBROID (10.3 X 9.9 X 8.9CM). HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT. HER VITALS WERE STABLE AT THE TIME OF ADMISSION. PAC CLEARANCE WAS TAKEN. SHE UNDERWENT TOTAL ABDOMINAL HYSTERECTOMY WITH BILATERAL SALPINGOOOPHERECTOMY ON 30/3/23. THERE WERE NO INTRAOP OR POSTOP COMPLICATIONS. PATIENT PASSED



STOOL AND URINE. POD 3 CHECK DRESSING WAS DONE, STITCH SITE HEALTHY. PATIENT CONDITION IS STABLE NOW AND HENCE BEING DISCHARGED WITH ADVICE TO FOLLOW UP IN GYNAEC OPD WITH HPE REPORT .

CONDITION AT DISCHARGE: GC = FAIR, AFEBRILE

PR = 80 BEATS PER MIN

RR = 16 BREATHS PER MIN

BP = 112/80 MM HG

SPO2 = 100 %

CNS CVS/RS = WNL

PA = SOFT, NONTENDER, STITCH SITE HEALTHY

LE = NO ACTIVE BLEEDING

ADVICE ON DISCHARGE:

HIGH PROTEIN DIET.

TAB CEFIXIME 200MG BD FOR 2 DAYS

TAB METRO 400MG TDS FOR 2 DAYS

TAB PANT 40 MG OD FOR 2 DAYS

TAB PCM 500MG TDS FOR 2 DAYS F/B SOS IF PAIN

TAB ONDEN 5 MG SOS IF VOMITING

TAB CALCIUM VITAMIN D3 BD X 3 MONTHS

ORAL CAFFEINE INTAKE TDS FOR 3 DAYS

TAB IRONFOLIC ACID 1 TAB PO OD FOR 3 MONTHS

TAB MVBC 1 TAB OD FOR 3 MONTHS

BATH REGULARLY

WASH STITCH SITE WITH SOAP WATER AND KEEP DRY

METRO P OINTMENT LOCAL APPLICATION BD

SEXUAL ABSTINENCE FOR 6 WEEKS

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 10-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : FOLLOW UP IN GYNAEC OPD ON 10/04/23 WITH HPE REPORT OR SOS IN CASE OF SEVERE PAIN ABDOMEN, BLEEDING PV, DISCHARGE FROM STITCH SITE OR FEVER

Summary Prepared By

anonymous

Summary Approved By

anonymous